RCM Prototype Presenter Guide

Presenter guide

Keep this simple. The product story is:

TatvaCare helps an RCM associate see what is stuck, understand what evidence is missing, and move the next claim step forward without jumping across email, folders, and spreadsheets.

What the three sidebar surfaces are for

1. Command Center

Use this as the opening view.

- Shows queue size, risk, SLA pressure, and value at stake
- Tells the client this is the morning operating view, not where detailed work happens
- Good for a 15-20 second setup only

2. Worklist

This is the main operational screen.

- Split into **Pre-auth**, **Concurrent**, and **Post-discharge**
- This is where the associate decides which case to open next
- For this demo, the visible cases are:
  - **Ibu Siti Aminah** — live pre-auth
  - **Ayu Lestari** — manual packet build from uploaded documents
  - **Reza Firmansyah** — concurrent stay extension watch
  - **Budi Santoso** — clean post-discharge success path
  - **Ravi Subramaniam** — partial settlement with appeal in flight

3. Payor Intelligence

Use this near the end.

- Shows payor correspondence and reusable payor rules
- Explains where learned rules come from
- Connects inbox signals back to what the associate sees inside the patient workspace

What the patient workspace is for

When you open a patient from the Worklist, there are two tabs:

- **Action** = what the associate should do now
- **Journey** = what has happened across the episode so far

For the live demo, spend most of the time in **Action** and use **Journey** only to support the story when needed.

Suggested click flow

1. Start at Command Center

1. Open `/`.

2. Say: *"This is the morning operating view. It shows where the queue is sitting, where value is at risk, and which stack needs attention first."*
3. Point to the queue split across **pre-auth**, **concurrent**, and **post-discharge**.
4. Move on quickly to **Worklist**.

2. Use Worklist to frame the queue

1. Open `/worklist`.
2. Say: *"This is the operator's main queue. It is organized by where the case sits in the revenue cycle."*
3. Click **Pre-auth** and point out:
    - **Ibu Siti Aminah** = connected EMR, live insurer review
    - **Ayu Lestari** = sparse-data case, packet built from uploaded scans
4. Click **Concurrent** and point out:
    - **Reza Firmansyah** = inpatient stay being monitored for extension and clinical support
5. Click **Post-discharge** and point out:
    - **Budi Santoso** = successful settled case
    - **Ravi Subramaniam** = contesting a partial settlement

3. Budi Santoso — clean EMR-connected success path

1. Open **Budi Santoso** from **Post-discharge**.
2. Say: *"This is the best-case path. The hospital already has the clinical record, the policy is on file, and the platform drives the packet cleanly."*
3. In the header, show:
    - BPJS payor
    - all three GL states approved
    - SLA met / settled in 6 days
4. In **Action**, point to:
    - **Policy-derived**
    - requirements already received
    - no blockers
5. In **Journey**, scroll to **Surgery** and show the scope expansion and top-up approval.

4. Ibu Siti Aminah — live pre-auth under review

1. Go back and open **Ibu Siti Aminah** from **Pre-auth**.
2. Say: *"This is a live case in flight. The point here is not appeal or denial recovery. The point is that the initial packet is already structured before the insurer comes back."*
3. Show in the header:
    - initial GL = submitted
    - top-up = not started
    - final = not started
    - amber SLA bar
4. In **Action**, point to:
    - **Policy-derived**
    - the current packet requirements
    - pending vs received evidence
5. Optional: in **Documents**, click **Upload document**, pick the antenatal note or anaesthesia clearance, then click **Upload selected**.

5. Reza Firmansyah — concurrent monitoring case

1. Go back to **Worklist** and click **Concurrent**.
2. Point to **Reza Firmansyah**.
3. Say: *"This queue is for patients already in hospital. The system is monitoring whether the current authorization still holds or whether an extension needs support."*
4. Show:
    - the case sits in the concurrent queue

- the extension is being drafted
- this is about active utilization management, not final billing yet

5. Do not stay here long. This is a queue-level proof point.

6. Ravi Subramaniam — partial settlement and appeal

1. Go back and open **Ravi Subramaniam** from **Post-discharge**.
2. Say: *"This is the recovery case. The hospital was paid partially, and the associate needs to push back with a stronger packet."*
3. Show in the header:
   - **Verdict — Approved/Rejected (contesting)**
   - final claim = partial
   - appeal SLA remaining
4. In **Action**, point to:
   - blockers tied to the appeal
   - **Case-learned** rule source
   - the appeal packet guidance
5. In **Journey**, scroll to **Discharge** and show the appeal evidence pack.

7. Ayu Lestari — manual packet build from uploaded documents

1. Go back and open **Ayu Lestari** from **Pre-auth**.
2. Say: *"This is the sparse-data case. The hospital has basic demographics, but the supporting evidence is coming in as outside documents."*
3. In **Action**, start in **Documents**.
4. Click **Upload document**.
5. First upload:
   - **OB-GYN referral**
   - **Pelvic MRI**
6. As they process, point to:
   - **Upload / Extract / OCR / Code / Packet**
   - OCR and coding signals in the document cards
   - requirement rows moving from **Pending** to **Received**
7. Second upload:
   - **Lab panel**
   - **Pre-op anaesthesia note**
8. Optional final upload:
   - **PRU policy PDF**
9. Then say: *"Once the policy is uploaded, the same case moves from case-learned logic to policy-derived logic."*

How to explain the rule engine

Keep this short.

- If the patient has an uploaded policy or benefits document, the workspace shows **Policy-derived** rules.
- If there is no patient-specific policy on file, the workspace shows **Case-learned** rules based on prior payor behavior and correspondence.
- The important point is not the label itself. The point is that the associate sees **what is required**, **what has been received**, and **what is still missing** before sending the packet.

Close

End with:

"This prototype is not trying to replace every RCM system. It is showing one simpler thing: give the associate a clear queue, a clear case workspace, and a clear evidence path from intake to submission or appeal."